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Project #65: Senior Care Navigator Program: Using Care Navigation to Improve Patient-Reported Outcomes Amongst Older Adult Patients

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AIM

- Problem Statement:** Older adults struggle to navigate health and social care systems, resulting in poorer patient health and satisfaction.
- Improvement (Goal) Statement:** Design and implement a patient navigation program to help older adult patients and their caregivers navigate the health and social care systems resulting in improved patient health and satisfaction.

PLAN: CURRENT STATE

- U.S. population is aging rapidly (21% of American population by 2020, up from 15%)
- Michigan population is aging faster than national average
- Declines in marriage, increases in divorce, and lower fertility mean that **more people reach older adulthood without a traditional caregiver** (spouse or adult child)
- Onus is on older adult patients to navigate complex health and social care systems on their own**
 - Schedule and keep medical appointments with multiple providers
 - Search for and obtain necessary health system and community help and resources
 - Self-advocate to ensure that the help and services that are received are adequate and sufficient
 - Self-management of their health

DO: CORRECTIVE ACTION

- Creation of Senior Care Navigator Program (SCNP), a free program for older adult patients (50 years or older), or their caregivers, who need support navigating health and social care systems (including information, advocacy, navigation, resources, self-management of health; Figure 1).
 - Establishment of multidisciplinary team: 2 registered nurses (Program Director and Manager) and 2 community health workers (CHW) certified in advanced care planning (Senior Navigators) to run the SCNP
 - SCNP Patient and Family Advisory Council (PFAC) to solicit feedback on the creation and operation of the SCNP
 - Senior Navigators (SNs) onboarding and training
 - Establish referral process in EPIC
 - Available at 3 HFMC (Commerce Township, Farmington, and Columbus Internal Medicine) and 1 Gerontology Clinic (HFWBH)

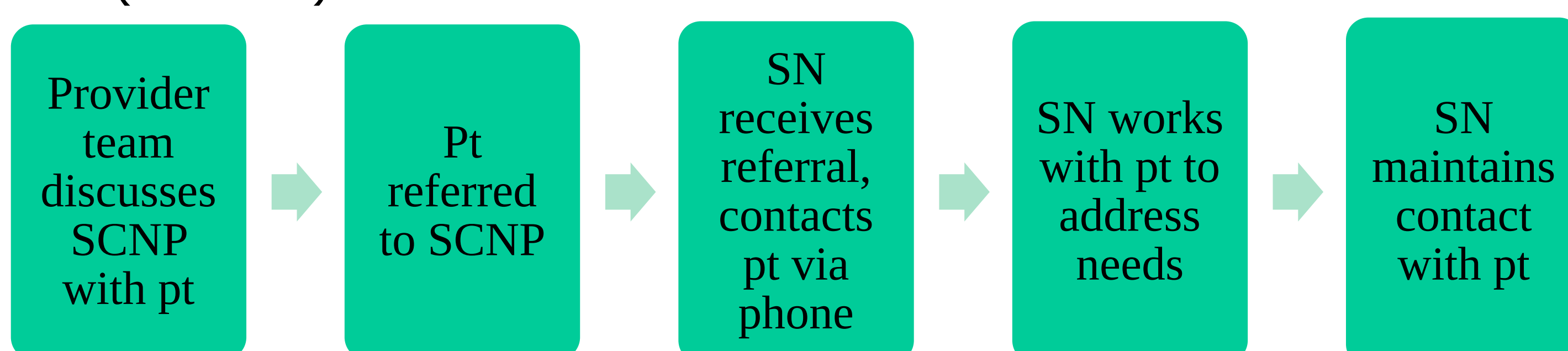


Figure 1. Overview of patient journey through Senior Care Navigator Program

CHECK (EVALUATION OF CHANGES)

- SCNP pilot launched in June 2021.
- Partnership with Department of Public Health Sciences to evaluate the program: 2 epidemiologists, 1 statistician, 1 trained interviewer.
- Patients may opt in (or not) to the SCNP's evaluation during their initial phone conversation with the SN.
- Patients complete baseline, as well as 3-, 6-, and 6-month follow up phone interviews with a trained interviewer to complete the following measures: EQ-5D health today (general health) and 4 PROMIS measures (emotional, depression, anxiety, self-efficacy).
- As of February 2023:
 - 301 patients have utilized the SCNP; 54 patients have completed or are participating in program evaluation.

MEASURES

- At 3-months: No differences from baseline.

Table 1. Comparison of participants' reports from baseline to 3-months.

Variable	Baseline	3 months	Change ¹	p-value
EQ5D health today	69.0 ± 26.3	70.8 ± 22.7	1.0 ± 28.5	0.810
PROMIS emotional	53.1 ± 8.0	54.3 ± 7.5	1.2 ± 8.2	0.328
PROMIS depression	51.6 ± 8.5	52.0 ± 9.7	0.4 ± 7.4	0.716
PROMIS anxiety	54.4 ± 9.4	52.0 ± 10.3	-2.5 ± 9.6	0.097
PROMIS self-efficacy	39.6 ± 8.1	38.7 ± 7.1	-0.8 ± 6.9	0.467

Note. ¹ p < .05

- At 6-months: significant improvement in the PROMIS depression and a trend for PROMIS anxiety.

Table 2. Comparison of participants' responses from baseline to 6-months.

Variable	Baseline	3 months	Change ¹	p-value
EQ5D health today	69.9 ± 29.6	78.1 ± 16.9	7.9 ± 26.7	0.110
PROMIS emotional	54.0 ± 8.4	55.3 ± 7.7	1.2 ± 9.9	0.491
PROMIS depression	51.0 ± 8.1	47.8 ± 8.0	-3.2 ± 8.5	0.042 ¹
PROMIS anxiety	52.9 ± 9.8	49.6 ± 8.6	-3.3 ± 10.3	0.079
PROMIS self-efficacy	40.1 ± 8.0	39.5 ± 7.3	-1.0 ± 7.7	0.471

Note. ¹ p < .05

- At 9-months: significant improvements in the PROMIS depression and EQ5D health today.

Table 3. Comparison of participants' responses from baseline to 9-months.

Variable	Baseline	3 months	Change ¹	p-value
EQ5D health today	68.9 ± 31.5	82.1 ± 14.1	13.0 ± 29.2	0.032 ¹
PROMIS emotional	54.2 ± 8.5	55.0 ± 7.9	0.8 ± 9.7	0.681
PROMIS depression	51.3 ± 8.3	48.2 ± 7.8	-3.2 ± 6.0	0.010 ¹
PROMIS anxiety	53.1 ± 10.2	51.8 ± 8.3	-1.4 ± 10.9	0.523
PROMIS self-efficacy	40.5 ± 8.3	40.8 ± 8.1	0.2 ± 8.7	0.927

Note. ¹ p < .05

ACT: SUSTAIN AND SPREAD

- Continue to show value added, look at ongoing data through 2024.
- Currently tracking all uses of the SCNP through the IRB but only doing the program evaluation with those who opt in.
- Look at costs, return on investment (ROI), and reduction of costs to healthcare system to show additional value of program.
- There is work going on to expand to more than 4 locations, particularly primary care clinics. Initial needs assessments show expansion to Macomb area would be next step.
- Address SDOH for older adults first. Focus on social determinants of health and work in continued collaboration with case management, doctors, nurses, medical assistants, and other advance practice providers to increase referrals.
- The State of Michigan is currently looking at ways to reimburse for CHW services (which all Senior Care Navigators are certified as) for time/effort billable through Medicaid and also through Medicare. Would need to work in collaboration with HFH CHEW team to bill for services to increase revenue and look at ROI.
- Continue to utilize Faith Community Network for collaboration and additional community resources.

ADDITIONAL PHOTOS OR VISUALS



KEYS TO SUCCESS/LESSONS LEARNED

- Integration into health system budget will allow for further expansion. Multi team approach to care is key.
- Seniors in active crisis (i.e. Diagnosis Cancer, Stroke, New Alzheimer, Dementia) tend not to want to participate in the follow-up research surveys as they are overwhelmed. Look at data to see why other patients opt out of the research surveys to re-evaluate patients who may not need continued follow up and/or services.
- Providing a \$25 gift card to participants for completing the program evaluation increased overall participation.
- Quarterly PDCA cycles of improvement: add opt out reason on research intake form, add all referrals to research database even if not participating.